

APOLLOCARE SMART HOSPITAL

Master Hospital Policies, Government Schemes & Patient Guidelines
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Category: POLICIES_AND_SCHEMES

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SECTION 1: GOVERNMENT HEALTH SCHEMES & CASHLESS INSURANCE EMPANELMENT

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1.1 Ayushman Bharat PM-JAY (Pradhan Mantri Jan Arogya Yojana)

- Coverage Amount: Up to ₹5,000,000 (₹5 Lakhs) per family per year for secondary and tertiary inpatient hospitalization.
- Cashless Facilities: 100% cashless hospitalization covering bed charges, nursing, surgeon fees, OT charges, ICU expenses, diagnostics, medicines, and implants.
- Eligible Beneficiaries: Families listed in SECC 2011 database or possessing a valid PM-JAY Ayushman Card / Golden Card.
- Covered Specialties: Oncology, Cardiology, Cardiac Surgery, Orthopedics, Joint Replacement, Nephrology, Urology, Neurosurgery, General Surgery, and Pediatrics.
- Required Documentation at Helpdesk:

1. Original PM-JAY Ayushman Card / Golden E-Card

2. Aadhaar Card / Voter ID / Govt Photo ID of Patient

3. Family Ration Card / SECC Household Verification Slip

- Verification & Admission Desk: Dedicated Ayushman Bharat Counter at Gate 2, Ground Floor, Building A (Operational 24/7).
- Post-Discharge Allowance: Covered package includes 15 days of post-discharge medicines and diagnostic follow-up.
- Non-Covered Items: Outpatient consultations (OPD), cosmetic surgeries, organ transplant donor search fees, and personal comfort items (television, attendant meals).

1.2 Central Government Health Scheme (CGHS)

- Target Group: Serving and retired Central Government employees, pensioners, freedom fighters, and eligible dependents.
- Admission & Referral Protocols:
- Non-Emergency Admissions: Requires valid CGHS Plastic Card and official referral letter from CGHS Wellness Center Medical Officer.
- Emergency Admissions: Cashless admission granted immediately upon presentation of CGHS Plastic Card without prior referral. Emergency memo issued within 24 hours to CGHS authorities.
- Empaneled Services: OPD, IPD, Diagnostics (MRI, CT, PET Scan, Ultrasound), Day Care Procedures, Dialysis, and Chemotherapy.
- CGHS Helpdesk: Room 104, 1st Floor, Building B (Open Monday–Saturday, 9:00 AM – 5:00 PM). Hotline: +91-11-2980-4401.

1.3 Employees' State Insurance Corporation (ESIC)

- Target Group: Insured Persons (IP) under ESI Act and registered dependents earning within statutory wage limits.
- Cashless Referral System: Requires Pehchan Card / e-Pehchan Slip, Form P1 / P2 referral issued by ESIC Dispensary/Hospital.

- Direct Emergency Admission: In case of acute trauma, myocardial infarction, or stroke, emergency admission is authorized without pre-referral upon IP verification within 48 hours.
- ESIC Desk: Counter 5, Main OPD Reception (Contact: +91-11-2980-4405).

1.4 State Health Insurance Schemes (Arogyasri & State CM Relief Funds)

- Coverage: Up to ₹5 Lakhs cashless coverage for BPL cardholders and state government health scheme beneficiaries.
- Covered Conditions: 1,050+ surgical and medical packages including polytrauma, renal failure, pediatric surgeries, and burn care.
- Free Post-Discharge Services: Includes 10 days of post-discharge medications and 1 free follow-up consultation within 30 days.

1.5 Private Health Insurance Companies & Third Party Administrators (TPAs)

- Empaneled Insurers & TPAs: Star Health, HDFC ERGO, ICICI Lombard, Niva Bupa, Care Health, Max Bupa, Bajaj Allianz, Reliance General, SBI General, Medi Assist TPA, Paramount TPA, Heritage TPA, Family Health Plan TPA, MDIndia TPA.
- Cashless Pre-Authorization Process:

1. Patient submits TPA Pre-Auth Form, Doctor's Admission Slip, Insurance Policy Card, and ID Proof at TPA Desk 3 hours prior to planned admission.

2. TPA Pre-authorization approval processed within 2 to 4 hours.

3. Security Deposit: A refundable deposit of ₹5,000 is collected at admission to cover non-payable consumables (gloves, masks, thermometer probes, tissues) as mandated by IRDAI regulations.

- Reimbursement Support: For non-empaneled insurers, complete itemized bills, discharge summary, diagnostic reports, and payment receipts are issued within 24 hours of discharge.
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SECTION 2: HOSPITAL ADMISSION, BILLING & REFUND POLICIES

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2.1 Admission Procedures

- Planned Admission: Doctor issues an Admission Request Slip. Patient reports to Admission Counter (Lobby, Building A) with Photo ID, Address Proof, and initial deposit or Pre-Auth approval.
- Emergency Admission: Patient admitted immediately through Emergency Department (Gate 1). Life-saving treatment initiated without demanding initial deposit.
- Room Categories: General Ward (6 Beds), Semi-Private Twin Sharing, Deluxe Private Room, Super Deluxe Suite, Intensive Care Unit (ICU/CCU/NICU).

2.2 Billing & Payment Guidelines

- Accepted Payment Modes: Cash, Credit Cards, Debit Cards, UPI (Google Pay, PhonePe, Paytm), Net Banking, NEFT/RTGS.
- Daily Interim Bills: Generated every evening at 6:00 PM for IPD patients and delivered to patient attendant or accessible via Patient Portal under "Billing".
- Final Billing & Discharge Audit: Takes approximately 2 hours after doctor signs final discharge summary to finalize pharmacy returns, diagnostic billing, and TPA final approval.

2.3 Refund & Cancellation Policy

- Outpatient Appointment Cancellation:
- Cancellation >2 hours prior to scheduled slot: 100% full refund initiated automatically to original payment method within 3 to 5 business days.
- Cancellation <2 hours or No-Show: 20% processing fee retained; 80% refunded or credited to patient wallet for future booking.

- Inpatient Advance Deposit Refund:
- If excess deposit remains after final bill settlement, refund is processed directly to patient's bank account via NEFT within 48 hours of discharge audit.
- Diagnostic Test Cancellation: Full refund granted if cancelled prior to sample collection or scan execution.

2.4 Financial Assistance & Subsidy Policy

- Economically Weaker Section (EWS) Concession: Patients with verified annual family income below ₹150,000 are eligible for 50% to 100% waiver on bed charges, nursing fees, and routine diagnostic tests.
- Application Process: Submit income certificate / Ration Card to Medical Social Welfare Department (Room 12, Ground Floor). Concessions sanctioned within 12 hours after social evaluation.
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SECTION 3: INPATIENT CARE, VISITING HOURS & SAFETY GUIDELINES

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3.1 Visiting Hours & Visitor Pass Policy

- General Wards & Private Rooms:
- Morning Hours: 10:00 AM – 1:00 PM daily
- Evening Hours: 4:30 PM – 8:00 PM daily
- Visitor Restriction: Maximum 2 visitors allowed per patient at any given time to maintain infection control and peaceful environment.
- Intensive Care Units (ICU, CCU, NICU, PICU):
- Visiting Time: 5:00 PM – 6:00 PM daily (Strictly 1 hour window).
- Visitor Limit: Exactly 1 adult visitor permitted per patient per session with specialized ICU Visitor Pass.
- Age Restriction: Children below 12 years are strictly prohibited inside ICUs for safety and immunity reasons.

3.2 Attendant Guidelines

- 1 continuous Attendant Pass is issued at the time of admission.
- Attendants must wear the issued badge visibly at all times inside hospital premises.
- Outside food, home-cooked perishable meals, flowers, and soft toys are strictly prohibited inside clinical wards and ICUs to prevent bacterial contamination.
- In-house clinical dietitians provide customized nutritious meals for all inpatients according to medical guidelines.

3.3 Infection Control & Hygiene Rules

- Mandatory hand sanitization using alcohol-based rubs before entering and leaving patient care zones.
- N95 or surgical masks mandatory in High-Risk Zones: Oncology, Transplant Wards, Respiratory Isolation, and ICUs.
- No smoking, chewing tobacco, or consuming alcohol anywhere on hospital campus (Strict legal penalty under Section 4 of COTPA Act).
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SECTION 4: EMERGENCY, TRAUMA & AMBULANCE PROTOCOLS

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4.1 24/7 Emergency & Trauma Care

- Location: Gate 1, Ground Floor, Building A (Direct Vehicle Access).

- Emergency Hotline: +91-11-2980-9999 / Toll-Free Ambulance Helpline: 108.
- Triage Category Protocols:

1. Red Category (Critical / Resuscitation): Cardiac arrest, polytrauma, stroke, severe respiratory failure. Immediate zero-wait intervention in Resuscitation Bay.

2. Yellow Category (Urgent): High fever, severe abdominal pain, fractures, deep lacerations. Evaluated within 15 minutes.

3. Green Category (Non-Urgent): Minor cuts, mild fever, chronic pain. Evaluated within 30 minutes.

4.2 Zero-Delay Stabilization Mandate

- No advance deposit or administrative clearance is required to initiate life-saving emergency medical stabilization.
- All emergency patients receive immediate medical assessment and initial stabilization care regardless of initial payment status.

4.3 Advanced Life Support (ALS) Ambulance Fleet

- Fleet Specification: Equipped with transport ventilators, multipara cardiac monitors, defibrillators, emergency medication kits, and trained paramedics.
- Service Radius: Available 24/7 within 30 km radius of the hospital. GPS tracking link sent via SMS upon booking.
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SECTION 5: OUTPATIENT (OPD), DIAGNOSTICS & PHARMACY GUIDELINES

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5.1 OPD Registration & Consultations

- Registration Fee: One-time registration fee of ₹200 issued with a unique UHID (Universal Health ID).
- Validity: OPD consultation fee covers 1 free follow-up review within 14 days with the same consulting specialist.
- Booking Channels: Online via ApolloCare Portal, Mobile App, or OPD Counter 1–8 (Building A Lobby).

5.2 Diagnostic Testing & Electronic Reports

- Location: Central Diagnostic Center, Building B, Basement 1.
- Turnaround Times:
- Routine Blood & Urine Biochemistry: 3 to 4 hours.
- Digital X-Ray: 1 hour.
- Ultrasound / Doppler: Immediate preliminary result; full radiologist report in 3 hours.
- CT Scan & MRI: 6 to 12 hours (Urgent emergency scans reported within 1 hour).
- Digital Access: All diagnostic reports and radiology images are automatically published to the Patient Portal under the "Lab Reports" section upon verification.

5.3 24/7 Pharmacy & Medicine Discount Rules

- Location: Ground Floor Main Entrance & Emergency Block.
 - Senior Citizen Discount: Flat 10% discount on all generic and branded medicines for senior citizens (age 60+) upon showing Aadhaar Card / Senior Citizen ID.
 - Mandatory Prescription Rule: Schedule H, H1, and X drugs (antibiotics, psychotropics, narcotics) will NOT be dispensed without a valid registered doctor's prescription.
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SECTION 6: PATIENT RIGHTS, RESPONSIBILITIES & GRIEVANCE REDRESSAL

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6.1 Patient Rights

1. Right to receive compassionate, respectful, and non-discriminatory care regardless of gender, caste, religion, or socio-economic status.
2. Right to complete information regarding diagnosis, proposed treatment options, risks, and estimated treatment cost.
3. Right to informed consent prior to any surgical procedure, anesthesia, or high-risk intervention.
4. Right to request a second medical opinion from another specialist.
5. Right to complete privacy and medical record confidentiality under DPDP Act 2023.
6. Right to receive copies of medical records and discharge summary within 72 hours of written application.

6.2 Patient Responsibilities

1. Provide truthful and complete medical history, past prescriptions, allergies, and pre-existing conditions.
2. Strictly follow the agreed clinical treatment plan and medical advice.
3. Respect hospital rules, visiting hours, quiet zones, and clinical staff.
4. **Zero-Tolerance Policy: Verbal abuse, physical assault, or intimidation of doctors, nurses, or hospital staff is a punishable criminal offense under the Healthcare Service Personnel Protection Act.**

6.3 Grievance Redressal & Escalation Mechanism

- Patient Care Officer: Room 101, Ground Floor Lobby (Mon–Sat, 9:00 AM – 6:00 PM).
 - Email Escalation: complaints@apollocare.org / patient.experience@apollocare.org.
 - Resolution Commitment: Operational grievances resolved within 24 hours; clinical quality reviews completed within 48 hours.
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SECTION 7: APPOINTMENT, SCHEDULING & CANCELLATION GUIDELINES

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7.1 Appointment Booking

- Patients may book appointments through the ApolloCare Patient Portal, Mobile App, Hospital Reception, or authorized hospital staff.
- Appointment booking requires the patient's UHID or registration details.
- Patients must provide accurate contact information for appointment confirmation and reminders.
- The hospital may send appointment reminders through SMS, email, or authorized digital communication channels.
- Patients should arrive at least 15 minutes before the scheduled consultation time for registration and verification.
- Emergency cases are not required to follow routine appointment scheduling procedures.

7.2 Appointment Confirmation

- An appointment is considered confirmed only after the system generates an appointment confirmation.
- Patients will receive the doctor name, department, date, time, location, and appointment reference number.

- Patients should verify appointment details before visiting the hospital.
- Appointment availability may change because of emergency duties, doctor schedules, or operational requirements.

7.3 Rescheduling

- Patients may request rescheduling through the Patient Portal, Mobile App, or Appointment Desk.
- Rescheduling is subject to doctor availability.
- The hospital may automatically suggest the next available appointment slot.
- If a doctor becomes unavailable, the hospital may provide an alternative appointment slot or an alternate specialist where appropriate.

7.4 Cancellation

- Patients may cancel appointments through authorized hospital channels.
- Refund eligibility will follow the hospital's applicable cancellation and refund policy.
- Repeated no-shows may result in temporary restrictions on advance appointment booking.
- Emergency situations may be considered separately by the Patient Care Department.
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SECTION 8: SMART HOSPITAL AI ASSISTANT POLICY

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8.1 Purpose of the AI Assistant

- The ApolloCare Smart Hospital Assistant is designed to help patients navigate hospital services and information.
- The AI Assistant may assist with appointment scheduling, hospital information, department navigation, policy-related questions, document organization, billing information, and general administrative support.
- The AI Assistant is intended to improve access to information and reduce routine administrative workload.

8.2 Medical Advice Limitation

- The AI Assistant must not independently diagnose diseases or medical conditions.
- The AI Assistant must not replace a qualified doctor, nurse, pharmacist, or other healthcare professional.
- Information provided by the AI Assistant must not be treated as a medical diagnosis or personalized treatment plan unless reviewed and explicitly provided by an authorized healthcare professional.
- When a patient's request requires clinical judgment, the AI Assistant must recommend consultation with an appropriate healthcare professional.
- In emergency situations, the AI Assistant must prioritize emergency instructions and direct the patient to the Emergency Department or emergency services.

8.3 AI Response Guidelines

- The AI Assistant must provide information based on authorized ApolloCare hospital data and approved knowledge sources.
- The AI Assistant must not invent doctors, appointments, hospital policies, insurance benefits, medical results, or patient records.
- When required information is unavailable, the AI Assistant must clearly state that the information is unavailable rather than generating an unsupported answer.
- The AI Assistant must distinguish between hospital policy information and general health information.
- The AI Assistant should use simple and understandable language whenever possible.

8.4 Human Escalation

- Patients may request assistance from hospital staff at any stage.
- The AI Assistant must escalate requests involving serious medical concerns, complaints, disputed bills, sensitive patient information, or complex clinical decisions to an appropriate hospital department.
- Patients must be provided with relevant contact information or escalation options whenever available.
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SECTION 9: PATIENT DATA PRIVACY & DIGITAL SECURITY

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9.1 Patient Information

- Patient information includes medical records, appointment details, contact information, insurance information, billing information, diagnostic reports, prescriptions, and other information associated with the patient's healthcare services.
- Patient information must be accessed only for legitimate healthcare, administrative, billing, insurance, or authorized support purposes.

9.2 Access Control

- Access to patient records is restricted according to authorized roles and responsibilities.
- Hospital employees must access only the information required to perform their assigned duties.
- Patients must not share their portal passwords, OTPs, authentication codes, or account credentials with unauthorized persons.
- Hospital staff will not request a patient's password or authentication credentials.

9.3 Patient Verification

- Identity verification may be required before providing sensitive patient information.
- The hospital may use UHID, registered mobile number, OTP, government-issued identification, or other approved verification methods.
- Sensitive information must not be disclosed to an unauthorized third party.

9.4 AI Data Access

- The Smart Hospital AI Assistant must access only the patient information necessary to complete the requested task.
- AI systems must not retrieve unrelated patient records.
- Patient information must not be exposed to another patient or unauthorized user.
- AI-generated responses must not reveal confidential information belonging to another individual.

9.5 Data Security

- Hospital systems must implement appropriate authentication, authorization, encryption, logging, monitoring, and access-control mechanisms.
- Suspicious access, unauthorized disclosure, or security incidents must be reported to the designated hospital security or IT department.
- Digital access logs may be maintained for security, auditing, and compliance purposes.
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SECTION 10: ELECTRONIC MEDICAL RECORD & PATIENT PORTAL POLICY

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10.1 Electronic Medical Records

- Authorized clinical and administrative information may be maintained electronically.
- Electronic records may include consultation notes, prescriptions, laboratory reports, radiology reports, discharge summaries, appointment history, and billing information.
- Corrections to medical records must follow the hospital's approved record-management procedures.

10.2 Patient Portal

- Patients may access authorized information through the ApolloCare Patient Portal.
- Available information may include appointments, bills, laboratory reports, prescriptions, discharge documents, and other approved records.

- Portal availability may depend on system maintenance and verification status.
- Patients should immediately report unauthorized account activity.

10.3 Digital Documents

- Digitally generated reports and documents must be treated as official hospital records when issued through authorized hospital systems.
- Patients should verify that documents belong to the correct UHID before sharing them with another person or organization.
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SECTION 11: DIAGNOSTIC REPORT & PRESCRIPTION GUIDELINES

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11.1 Diagnostic Reports

- Diagnostic reports must be reviewed by the appropriate authorized healthcare professional.
- Preliminary results may differ from final verified reports.
- Patients should rely on the final authorized report for official documentation.
- The AI Assistant may help patients understand the structure or terminology appearing in a report but must not independently provide a medical diagnosis.

11.2 Prescription Information

- Prescriptions must be issued or approved by authorized healthcare professionals.
- The AI Assistant may explain basic prescription instructions when the information is clearly available in an authorized record.
- The AI Assistant must not independently change medication dosage, frequency, duration, or prescription status.
- Patients must contact their doctor or pharmacist if they have questions about changing or stopping medication.

11.3 Medication Safety

- Patients should inform healthcare professionals about allergies, current medicines, supplements, and previous adverse reactions.
- Patients must not share prescription medicines with another person.
- Medication errors or suspected adverse reactions should be reported to the appropriate healthcare professional immediately.
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SECTION 12: EMERGENCY & HIGH-RISK REQUEST HANDLING

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12.1 Emergency Identification

The AI Assistant should recognize requests indicating potentially serious or emergency situations, including but not limited to:

- Severe chest pain
- Difficulty breathing
- Loss of consciousness
- Severe bleeding
- Suspected stroke
- Serious injury or trauma
- Severe allergic reaction
- Seizure
- Sudden severe weakness
- Other situations requiring immediate medical attention

12.2 Emergency Response

- The AI Assistant must not delay emergency care by conducting lengthy conversations.
- The patient should be directed to the Emergency Department or appropriate emergency services.
- The hospital emergency contact information should be provided when available.
- If the patient is already inside the hospital, the AI Assistant should direct them to the Emergency Department or appropriate clinical staff.

12.3 High-Risk Decisions

- The AI Assistant must not independently authorize surgery, discharge, medication changes, clinical procedures, or other high-risk medical decisions.
- Such decisions must be handled by authorized healthcare professionals.
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SECTION 13: BILLING, INSURANCE & PAYMENT ASSISTANCE

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13.1 Billing Information

- Patients may request information about available bills, payment status, deposits, refunds, and applicable hospital charges.
- Final charges are subject to verification by the appropriate billing department.
- The AI Assistant may explain available billing information but must not alter financial records without authorized approval.

13.2 Insurance Information

- Insurance eligibility and coverage are subject to the patient's policy terms, insurer approval, and hospital verification.
- The AI Assistant must not guarantee insurance approval.
- Pre-authorization decisions are made by the applicable insurer or TPA.
- Patients should contact the Insurance/TPA Desk for disputed claims or coverage questions.

13.3 Payment Security

- Patients must use only authorized hospital payment channels.
- Patients should not share card PINs, OTPs, banking passwords, or other confidential payment credentials with hospital staff or AI systems.
- Suspicious payment requests must be reported immediately to the hospital.
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SECTION 14: VISITOR, ATTENDANT & PATIENT CONDUCT POLICY

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14.1 Visitor Conduct

- Visitors must follow hospital visiting hours and visitor limits.
- Visitors must carry and display the appropriate visitor pass where required.
- Visitors must maintain a quiet and respectful environment.
- Photography, video recording, or audio recording of patients, staff, or clinical procedures may require authorization.

14.2 Restricted Items

- Weapons, hazardous substances, unauthorized medicines, alcohol, tobacco products, and other prohibited items are not permitted inside restricted hospital areas.
- Security personnel may conduct checks in accordance with hospital policy and applicable law.

14.3 Patient Conduct

- Patients and attendants must treat doctors, nurses, technicians, administrative employees, security personnel, and other patients respectfully.
- Threatening, abusive, violent, or disruptive behavior is prohibited.
- Serious incidents may be escalated to hospital security and appropriate authorities.
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SECTION 15: CHILDREN, ELDERLY PATIENTS & ACCESSIBILITY GUIDELINES

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15.1 Children

- Children visiting the hospital must be accompanied by a responsible adult.
- Children may be restricted from entering high-risk clinical areas.
- Pediatric patients should be directed to the designated pediatric services where applicable.

15.2 Elderly Patients

- Elderly patients may request wheelchair assistance, mobility assistance, or additional support during registration and hospital navigation.
- Priority assistance may be provided according to hospital operational procedures.

15.3 Patients With Disabilities

- The hospital will provide reasonable accessibility support within available facilities.
- Wheelchairs, ramps, accessible entrances, elevators, and assistance services may be provided where available.
- Patients requiring special assistance should inform hospital staff in advance whenever possible.

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SECTION 16: LANGUAGE & COMMUNICATION GUIDELINES

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16.1 Multilingual Support

- The Smart Hospital Assistant may support multiple languages where technically available.
- Patients may request communication in their preferred supported language.
- Medical terminology should be explained in simple language whenever possible.

16.2 Communication Accuracy

- Important appointment, billing, medication, or clinical information should be verified against the authorized hospital record.
- Translated information should not be treated as a replacement for an official medical document.
- Patients may request assistance from hospital staff when translation of important medical information is required.

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SECTION 17: PATIENT FEEDBACK & SERVICE QUALITY

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17.1 Feedback

- Patients may submit feedback regarding appointments, hospital services, staff interactions, facilities, billing, digital services, or other hospital experiences.
- Feedback may be submitted through the Patient Portal, Mobile App, Helpdesk, email, or authorized feedback forms.

17.2 AI Assistant Feedback

- Patients may report incorrect, incomplete, confusing, or inappropriate AI responses.
- AI-related complaints should be reviewed by the designated hospital technology or patient-experience team.
- Reported errors may be reviewed for system improvement.

17.3 Service Improvement

- Hospital departments may periodically review patient feedback and operational data to improve service quality.
- Patient feedback must be handled in accordance with applicable privacy and confidentiality requirements.
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SECTION 18: CYBERSECURITY & ACCOUNT SAFETY

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18.1 Account Security

- Patients are responsible for protecting their account credentials and registered devices.
- Patients should use strong passwords and avoid sharing credentials.
- OTPs and authentication codes must not be shared with another person.

18.2 Suspicious Activity

Patients should immediately report:

- Unauthorized login
- Unknown appointment changes
- Unexpected billing transactions
- Suspicious messages or links
- Unauthorized access to medical records
- Lost or stolen devices used to access the Patient Portal

18.3 System Availability

- Digital services may occasionally become unavailable because of maintenance, technical failures, network issues, or security incidents.
- During service interruptions, patients may use authorized offline hospital services such as reception, helpdesks, or emergency services.
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SECTION 19: THIRD-PARTY SERVICES & EXTERNAL COMMUNICATION

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19.1 Third-Party Services

- Certain hospital services may involve authorized insurers, TPAs, laboratories, pharmacies, payment providers, ambulance services, or technology providers.
- Information shared with third parties must be limited to information necessary for the authorized service.

19.2 External Communication

- Patients should verify that communications claiming to represent ApolloCare originate from authorized hospital channels.
- The hospital will not request confidential passwords, banking PINs, or authentication credentials through unauthorized communication channels.
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SECTION 20: AI AGENT TOOL & ACTION CONTROL POLICY

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20.1 Tool Authorization

- AI agents may use only tools authorized for their assigned responsibilities.
- An appointment agent should not access unrelated clinical records unless explicitly authorized.
- Agents must not access, modify, or delete data outside their permitted scope.

20.2 Confirmation Before High-Impact Actions

The AI Assistant must obtain explicit patient confirmation before performing high-impact actions such as:

- Booking an appointment
- Cancelling an appointment
- Rescheduling an appointment
- Initiating a refund request
- Sharing a medical document
- Submitting an insurance-related request
- Making other irreversible or financially significant changes

20.3 Tool Failure

- If a hospital API, database, MCP service, or other connected system fails, the AI Assistant must inform the patient that the requested action could not be completed.
- The AI Assistant must not claim that an action was completed when the external system did not confirm successful execution.

20.4 Audit Trail

- Important AI-initiated actions may be recorded in an audit log.
- Audit records may include the requested action, timestamp, authorized user/session, tool used, and action result.
- Audit logs must be protected from unauthorized modification.

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SECTION 21: DOCUMENT UPLOAD & FILE HANDLING GUIDELINES

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21.1 Supported Documents

Patients may be allowed to upload:

- Prescriptions
- Laboratory reports
- Radiology reports
- Discharge summaries
- Referral letters
- Insurance documents
- Other authorized healthcare documents

21.2 Document Verification

- Uploaded documents may be checked for readability, file integrity, and supported format.
- If a document cannot be read reliably, the patient may be asked to upload a clearer copy.
- The AI Assistant must not assume missing information from an unreadable document.

21.3 Document Privacy

- Uploaded documents containing personal or medical information must be treated as confidential.

- Documents must not be displayed to unauthorized users.
- Patients should avoid uploading documents that are unrelated to the requested hospital service.
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SECTION 22: CONSENT & COMMUNICATION PREFERENCES

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22.1 Patient Consent

- Patients may be required to provide consent before certain medical procedures, data-sharing activities, or services.
- Consent must be obtained through an authorized process.
- AI-generated information must not be treated as a substitute for formal medical consent.

22.2 Communication Preferences

Patients may specify available preferences for:

- SMS notifications
- Email notifications
- Appointment reminders
- Digital reports
- Billing notifications
- Other authorized hospital communications

22.3 Withdrawal of Optional Consent

- Patients may withdraw optional communication or data-sharing permissions where applicable.
- Withdrawal of consent may affect the availability of certain optional services.
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SECTION 23: AI TRANSPARENCY & RESPONSIBLE USE

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23.1 AI Disclosure

- Patients should be informed when they are interacting with an AI-powered hospital assistant.
- The AI Assistant must not represent itself as a human healthcare professional.

23.2 AI Limitations

- AI responses may contain errors or misunderstandings.
- Patients should verify important medical, financial, insurance, and appointment information through authorized hospital channels.
- The AI Assistant must clearly communicate uncertainty when sufficient information is not available.

23.3 No Fabrication

The AI Assistant must never:

- Invent patient records
- Invent appointments
- Invent medical results
- Invent insurance approvals
- Invent doctors or departments
- Invent hospital policies

- Claim that a tool or API was used when it was not used
- Claim that an appointment, cancellation, refund, or other action succeeded without confirmation
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SECTION 24: POLICY UPDATES & VERSION CONTROL

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24.1 Policy Updates

- Hospital policies may be updated because of operational changes, regulatory requirements, technology changes, or service improvements.
- Patients should refer to the latest officially published version of the applicable policy.

24.2 AI Knowledge Updates

- The Smart Hospital AI Assistant must use the latest approved hospital policy and service information available to it.
- Outdated information identified in the AI system should be reported to the designated hospital administrator or technology team.

24.3 Version Identification

Every official policy document should contain:

- Document ID
- Version number
- Effective date
- Approval authority
- Applicable department
- Review date where applicable
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SECTION 25: AI ASSISTANT ESCALATION MATRIX

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The Smart Hospital Assistant should route requests to the appropriate department as follows:

PATIENT APPOINTMENT

→ Appointment Desk

MEDICAL QUESTION REQUIRING CLINICAL JUDGMENT

→ Doctor / Clinical Department

EMERGENCY OR LIFE-THREATENING SITUATION

→ Emergency Department

INSURANCE APPROVAL / CLAIM

→ Insurance / TPA Desk

BILLING DISPUTE

→ Billing Department

REFUND ISSUE

→ Billing / Finance Department

MEDICAL RECORD REQUEST

→ Medical Records Department

PRIVACY OR DATA ACCESS ISSUE

→ Privacy / Data Protection / IT Department

AI SYSTEM ERROR

→ AI Support / Technology Team

PATIENT COMPLAINT

→ Patient Care Officer / Grievance Department

SECURITY INCIDENT

→ Hospital Security / IT Security Team

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SECTION 26: SMART HOSPITAL ASSISTANT — GENERAL RESPONSE RULES

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The AI Assistant must follow these principles for every patient interaction:

- 1. Understand the patient's request before taking action.**
- 2. Use the appropriate specialized agent or authorized tool for the requested task.**
- 3. Verify patient identity before accessing sensitive information.**
- 4. Access only the information required to complete the request.**
- 5. Never fabricate information.**
- 6. Never claim that an action was completed without confirmation from the relevant hospital system.**
7. Ask for confirmation before high-impact or irreversible actions.
8. Escalate clinical, emergency, financial, privacy, and grievance-related issues when human intervention is required.
9. Protect patient confidentiality.
 10. Provide clear and understandable explanations.
 11. Clearly distinguish general information from professional medical advice.
 12. Use the latest approved hospital policies and information.
 13. If information is unavailable or uncertain, state that clearly.
 14. Maintain an appropriate audit trail for authorized high-impact actions.
 15. Treat every patient fairly and respectfully without discrimination.
 16. Never expose information belonging to another patient.
 17. Do not make independent medical diagnoses or treatment decisions.
 18. Do not modify prescriptions, medical records, billing records, or appointments without appropriate authorization.
 19. Provide emergency guidance immediately when a potentially life-threatening situation is identified.
 20. Always provide a human-support option when the patient requests assistance or when the AI cannot safely complete the request.

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END OF ADDITIONAL POLICIES & PATIENT GUIDELINES